

1. Administrative & Study Identification

Full Study Title: A Study of BMS-986488 as Monotherapy and Combination Therapy in Participants With Advanced Malignant Tumors
Short Title/Acronym: BMS-986488 in Advanced Malignant Tumors
Protocol Number: CA234-0001
NCT Number: NCT06764771
Posting ID: 643302788647070897 **Sponsor Name / Company:** Bristol-Myers Squibb
Investigational Product: BMS-986488 (monotherapy or in combination with Adagrasib, Cetuximab, or Nivolumab)
Phase of Development: Phase 1 (PHASE1) **Trial Status:** RECRUITING (Currently recruiting participants) **Medical Monitor:** Bristol-Myers Squibb Clinical Director

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To determine if the experimental treatment with BMS-986488, alone or in combinations, is safe, tolerable, and has anti-cancer activity in patients with advanced malignant tumors. **Secondary Objectives:** To evaluate preliminary efficacy (e.g., Objective Response Rate) and pharmacokinetics of BMS-986488.

2.2 Background & Rationale To address the unmet medical need for patients with advanced, unresectable, or metastatic solid tumors that have progressed on standard therapies. This study investigates whether the novel agent BMS-986488, administered alone or with targeted/immunotherapies (such as Adagrasib for KRAS G12C mutated tumors), can offer safe and improved anti-cancer activity.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm / Sequential Assignment **Blinding:** Open-label (None) **Randomization:** Non-Randomized

3.2 Planned Enrollment & Duration Target \$N\$: 437 participants **Number of Sites:** Global Multicenter **Estimated Study Start:** 25-Mar-2025 (Actual Start Date) **Estimated Primary Completion:** 15-Oct-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participant must be ≥ 18 years of age. 2. Histologically confirmed diagnosis of a locally advanced and unresectable or metastatic solid tumor malignancy with any of the following tumor types: * **Part 1A:** clear-cell renal cell carcinoma (ccRCC), clear-cell ovarian cancer (ccOC), non-small cell lung cancer (NSCLC), colorectal cancer (CRC), and pancreatic ductal adenocarcinoma (PDAC). * **Parts 2A, 1D, 2D:** ccRCC. * **Part 1B:** solid tumors with KRAS G12C mutation. * **Part 2B:** NSCLC with KRAS G12C mutation. * **Parts 1C, 2C:** colorectal cancer (CRC) with KRAS G12C mutation. 3. Participants must have measurable disease per RECIST v1.1 and an Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 4. **Phase Criteria:** PHASE_REQUIREMENT (Trial must be in PHASE1).

4.2 Exclusion Criteria 1. Untreated central nervous system (CNS) metastases or Leptomeningeal metastasis (carcinomatous meningitis). 2. Impaired cardiac function or clinically significant cardiac disease. 3. For Parts 1B, 1C, 2B, 2C only (combination with adagrasib): i) History of pneumonitis or interstitial lung disease (ILD). ii) History of prior severe cutaneous adverse reactions (SCARs), including Stevens-Johnson syndrome (SJS) and toxic epidermal necrolysis (TEN). 4. Other protocol-defined inclusion/exclusion criteria apply.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: BMS-986488 administered at escalating and expansion dose levels, either as monotherapy or combined with Adagrasib, Cetuximab, or Nivolumab. (*Note on Cetuximab: Preferred Name: cetuximab; Trade Name: Erbitux; ATC Code: L01FE01; EMA URL: https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf*) **Route of Administration:** Intravenous (IV) and/or Oral (PO) based on assigned combination agent. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for cancer patients. **Prohibited:** Concurrent investigational drugs or other systemic anti-cancer therapies outside the protocol.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Imaging (RECIST v1.1), Safety Labs
Baseline	Day 0	First Dose, PK/PD sampling, Safety Assessments
Interim	Q2W - Q4W	Safety Labs, Efficacy Assessment (Scans)

every 8-12 weeks), AE Evaluation | | **End of Study** | Follow-up | Safety follow-up up to approximately 100 days after the last dose, Survival follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Number of participants with Adverse Events (AEs), number of participants with Serious Adverse Events (SAEs), and number of participants with AEs meeting protocol-defined Dose-Limiting Toxicity (DLT) criteria.

Measurement Method: Clinical observation, vital signs, and laboratory assessments tracked until the end of the Safety Follow-up period (up to approximately 100 days after the last dose).

7.2 Secondary & Exploratory Endpoints * Preliminary anti-cancer efficacy as measured by Objective Response Rate (ORR), Duration of Response (DOR), and Progression-Free Survival (PFS). * Pharmacokinetic (PK) profile of BMS-986488, specifically: Maximum observed plasma concentration (\$C_{max}\$), Time of maximum observed concentration (\$T_{max}\$), and Area under the concentration-time curve in 1 dosing interval (\$AUC(TAU)\$).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring at each study visit and tracked for up to 100 days post-treatment.

Serious Adverse Events (SAEs): Reported within 24 hours to the sponsor and regulatory authorities. **Data Monitoring Committee (DMC):** An internal Dose Escalation Committee (DEC) will review safety data between cohorts to establish Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose (RP2D).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients receiving \$\ge 1\$ dose); DLT-evaluable Population; Efficacy Evaluable Set. **Sample**

Size Justification: \$N=437\$, based on standard clinical algorithms for Phase 1 dose-escalation and subsequent Phase 1b expansion cohorts. **Handling of Missing Data:** Missing safety data will generally not be imputed; efficacy data will be censored at the last evaluable tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Standard oncology early-phase monitoring

including regular onsite and remote data verification. **Audit Trail:** Electronic Case Report Form (eCRF) utilized with strict data query, resolution, and locking protocols.

11. Study Identifiers & Governance

Primary ID: CA234-0001 **Registry Number:** NCT06764771 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** A Study of BMS-986488 as Monotherapy and Combination Therapy in Participants With Advanced Malignant Tumors **Official Regulatory Title:** A Phase 1/1b Open-label Study of BMS-986488 as Monotherapy and Combination Therapy in Participants With Advanced Malignant Tumors

12. Clinical Framework & Ontology Metadata (Oncology Specific Adaptation)

Primary Condition / Disease Area: Advanced Malignant Tumors (N/A for MeSH Code) **Disease Preferred Name:** Advanced Malignant Germ Cell Tumor **Preferred UMLS Name:** Metastatic Cancer **Semantic Type:** Neoplastic Process **SNOMED ID:** 108344002 **SNOMED Hierarchy:** 254133009 | Metastatic malignant neoplastic disease | Metastatic malignant neoplastic disease (disorder) **Definition:** A malignant germ cell tumor that has spread extensively to other anatomic sites or is no longer responding to treatment. **Source Level:** 5 **Medical Methodology:** Targeted Monotherapy and Combination Therapy (with PD-1, EGFR, or KRAS inhibitors) **Optimization Target:** Dose escalation to establish safety profile and Recommended Phase 2 Dose (RP2D)

13. Study Procedures & Methodology

Management Pathway: Standard early-phase oncology clinical pathway **Education Protocol (HCP):** RECIST v1.1 imaging training, dose-limiting toxicity (DLT) reporting criteria **Education Protocol (Patient):** Informed consent review, AE symptom reporting, and strict contraceptive guidance **Quality Audit Mechanism:** Continuous safety data reviews prior to dose escalation

14. Observation & Follow-Up Schedule

Total Duration: Estimated 2025 to 2027 (Until disease progression or end of follow-up) **Onsite Visit Cadence:** Every cycle (typically 2 to 4 weeks depending on the assigned treatment arm) **Remote Monitoring Frequency:** Between cycles and during the survival follow-up period **Checklist Review Interval:** Regular tumor assessments every 8-12 weeks

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					